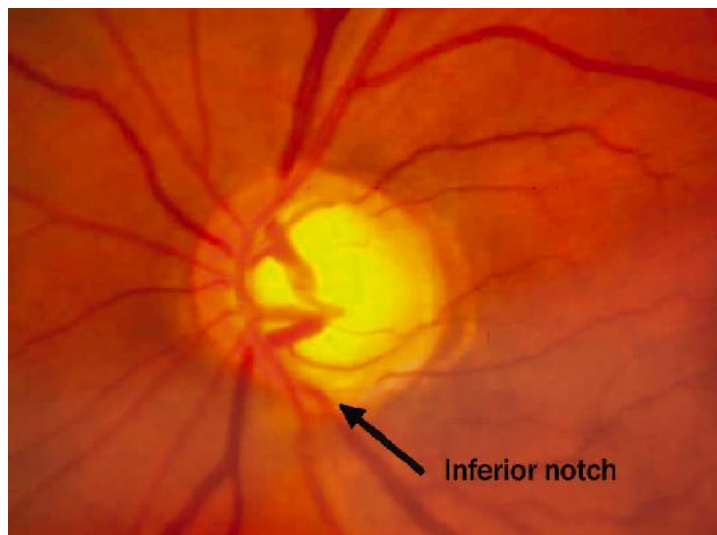


## Correlate rim loss with field loss

The location of neuroretinal rim damage helps predict the corresponding visual field defect.



The original photograph marks an inferior neuroretinal rim notch. Inferior structural damage corresponds to superior field loss; correlate the disc with reproducible perimetry rather than diagnosing disease from the arrow or cup size alone.

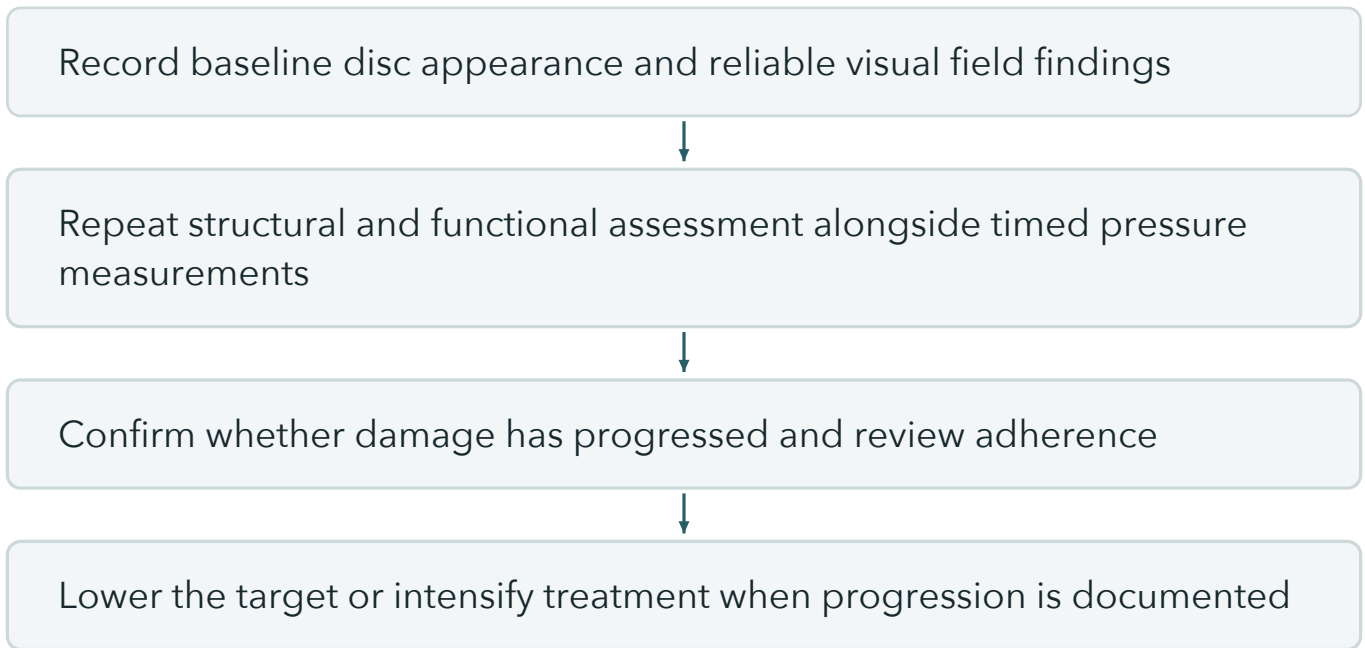
## Read the disc in context

Look for pathological cupping, focal rim loss, peripapillary changes and nerve fibre layer abnormalities. Rim thickness normally follows inferior, superior, nasal then temporal (ISNT), but this pattern lacks specificity to stand alone. Cup asymmetry warrants scrutiny; documented enlargement matters. Superior and inferior arcuate defects reflect vulnerable arcuate fibre bundles. A nasal step reflects unequal loss above and below the horizontal raphe. Normal visual acuity can coexist with these defects because the central letter-chart task samples a different part of visual function.

# Use repeatable change to guide treatment

Follow-up compares the disc and field over time rather than checking only whether IOP is within a usual range.

## Assess progression



## Interpret the tests together

Stereo disc photography and OCT support structural monitoring; perimetry assesses function. A single pressure reading can miss fluctuation. Targets reflect existing damage and patient factors, and should be reconsidered if damage progresses despite apparently adequate control. As reserve diminishes, each further loss matters more. Preserved acuity or lack of symptoms should not overrule reproducible disc or field deterioration; laser or surgery may be needed when conservative treatment is insufficient.